Trastuzumab deruxtecan (Enhertu) · every 3 weeks

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

Make treatment fit into daily life

The first infusion takes 90 minutes. Later infusions may take 30 minutes if the previous infusion was tolerated. Allow extra time for assessment, anti-sickness medicines and any observation. A cannula or an existing central line can be used according to the clinical plan; there is no home pump.

Before each visit

Have the requested blood tests and report any new breathing symptoms before attending for treatment. Bring your medication list and the Enhertu alert card. Ask when your next heart scan is due.

On infusion day

Take the prescribed anti-sickness medicines as instructed. The infusion is longer the first time. Tell the nurse immediately about flushing, chills, rash, dizziness, wheeze or breathing difficulty during treatment.

Between infusions

Nausea can continue after treatment day. Follow the scheduled and rescue anti-sickness plan, drink regularly and contact the team early if vomiting prevents fluid intake. Keep activity manageable and ask for help with fatigue.

Scans answer different questions

Echo/multigated acquisition (heart pumping) scan (MUGA) monitors heart pumping function. Cancer-response scans follow the oncology plan. A computed tomography (CT) requested urgently for new cough or breathlessness investigates a possible treatment complication; it must not wait for a routine response scan.

If you already have a port

A trained nurse accesses and removes the port needle for the infusion visit, then flushes the line according to local policy. Report redness, pain, swelling or fever. A new port is an individual decision, not compulsory for everyone receiving Enhertu.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00776 v3b · official regimen
- EMA Enhertu product information · retrieved 25 October 2026
- Cortés et al. Nature Medicine2024 · DESTINY-Breast03 long-term analysis
- Modi et al. NEJM2022 · DESTINY-Breast04
- Pan-Asian adapted ESMO early breast cancer guidelines
- Pan-Asian adapted ESMO metastatic breast cancer guidelines
- ESO-ESMO ABC 5 consensus: tumour-marker interpretation
- DESTINY-Breast03: long-term survival analysis
- DESTINY-Breast04: previously treated HER2-low advanced breast cancer
- DESTINY-Breast03 long-term analysis (2024)
- DESTINY-Breast04 long-term analysis (2025)

Source reconciled; independent clinical, pharmacy and patient/PPI review pending